

CILE Clinical Consultation Report

Patient Vitals & Demographics

Age: 78 | Gender: F

Weight: weight was not given kg | SrCr: SrCr was not given mg/dL

Estimated CrCl: CrCl could not be calculated mL/min

Checking for DRUG-DRUG INTERACTIONS

[CRITICAL] SIMVASTATIN & ITRACONAZOLE: [CONTRAINDICATED] CYP3A4 Inhibition: massive increase in simvastatin AUC. Risk of fatal rhabdomyolysis and acute renal failure.

Predictive Clinical Alerts & Profiles

Drug: Metformin

[Drug Classification]: RENAL

[RoA]: IV, Oral, SubQ

[PGx Markers Found] Targets: CYP2C9, CYP3A4, CYP2C8

> [CLINICAL INTERCEPT] Monitor eGFR; contraindicated if eGFR <30 mL/min due to risk of lactic acidosis; do not initiate if eGFR 30-45.

> [!] CRITICAL: Renal-based dose adjustments were skipped due to missing vitals.

> [FDA ALERT] 8.5 Geriatric Use ZITUVIMET In general, dose selection for an elderly patient should be cautious, usually starting at the low end of the dosing range, reflecting the greater frequency of decreased hepatic, renal, or cardiac function, and of concomitant disease or other drug therapy and the higher risk of lactic acidosis

> [FDA ALERT] Renal function should be assessed more frequently in elderly patients.

Drug: Aspirin

[Drug Classification]: UNCLASSIFIED

[RoA]: IV, Oral

[PGx Markers Found] Targets: None identified

> [DEPRESCRIBE]: Per STOPP & CaDeN guidelines, avoid Aspirin for PRIMARY prevention in age > 70.

> [!] CRITICAL: Renal-based dose adjustments were skipped due to missing vitals.

Drug: Simvastatin

[Drug Classification]: BEERS, RENAL

[RoA]: Oral, SubQ

[PGx Markers Found] Targets: CYP3A4, RECEPTOR

> [CLINICAL INTERCEPT] Avoid doses >20mg in severe renal impairment (CrCl <30 mL/min). Advanced age (>=65) increases risk of rhabdomyolysis.

> [CLINICAL NOTE]: Statin for SECONDARY prevention (Post-MI/Stroke) should be continued regardless of age if benefits outweighs the risks UNLESS terminal.

> [!] CRITICAL: Renal-based dose adjustments were skipped due to missing vitals.

Drug: Itraconazole

[Drug Classification]: HEPATIC, CARDIAC

[RoA]: IV, Oral, SubQ

[PGx Markers Found] Targets: RECEPTOR, CYP3A4, METABOLIZER, CYP2D6

> [CLINICAL INTERCEPT] Potent CYP3A4 inhibitor. Potential for serious hepatotoxicity. Use with caution in patients with history of CHF.

> [!] CRITICAL: Renal-based dose adjustments were skipped due to missing vitals.

> [FDA ALERT] In general, it is recommended that the dose selection for an elderly patient should be taken into consideration, reflecting the greater frequency of decreased hepatic, renal, or cardiac function, and of concomitant disease or other drug therapy

> [FDA ALERT] Several of these reports included concurrent administration of quinidine which is contraindicated .

Drug: Gliclazide

[Drug Classification]: UNCLASSIFIED

[RoA]: Unspecified

[PGx Markers Found] Targets: N/A

> [!] CRITICAL: Renal-based dose adjustments were skipped due to missing vitals.